

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
IPAA Discrepancy		☒
Prior Malignancy History		☒
Local/Synchronous Primary Notes		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 3/23/12	

UUID: 784D0924-C38E-4C85-914B-B7F123F3E3C5
TCGA-EL-A3TA-01A-PR

Redacted



Pathology

port

Pathology,
Fax:

DOB:

Physician:

Sex:

Accession

Collected

Received:

Case type: Surgical Case

***** MODIFIED REPORT - REVIEW ADDENDUM SECTION *****

DIAGNOSIS

ICD-O-3
carcinoma, papillary, thyroid
8260/3

(A) CONFIRM PARATHYROID:

Parathyroid tissue, no tumor.

Site: thyroid, NOS C73.9 4-4-12
RD

(B) THYROID AND LEFT PARATRACHEAL TISSUE:

PAPILLARY CARCINOMA OF THYROID, LEFT LOBE (3.5 CM), TUMOR CONFINED TO THE THYROID.
MARGINS OF RESECTION FREE.

ISTHMUS:

Chronic inflammation.

Three lymph nodes, one with xanthogranuloma. (see comment)

RIGHT LOBE OF THYROID:

Adenomatoid nodule.

Twelve lymph nodes, no tumor.

(C) RIGHT NECK SEBACEOUS CYST:

Ruptured sebaceous cyst.

Entire report and diagnosis completed by

COMMENT

Immunoperoxidase were requested from the lymph nodes of the isthmus. Supplemental report will follow.

GROSS DESCRIPTION

(A) CONFIRM PARATHYROID - A tan-brown, ovoid fragment of soft tissue (0.5 x 0.2 x 0.2 cm, 0.0044 grams weight). The specimen is bisected, touch preps were performed and entire specimen submitted for frozen section in A.

*FS/DX: PARATHYROID TISSUE PRESENT.

(B) THYROID AND LEFT PARATRACHEAL TISSUE - A specimen of thyroid (right lobe 6.0 x 2.5 x 1.0 cm, left lobe 7.5 x 3.0 x 2.0 cm and isthmus 2.2 x 1.0 x 1.0 cm) was portion of soft tissue inferior to the left lobe (4.5 x 2.0 x 1.0 cm).

The inferior half of the left lobe has a 4.0 x 2.5 x 2.0 cm circumscribed nodule with a smooth surface. Cut sectioning reveals a solid and papillary tumor with focal hemorrhage. The tumor abuts the surface at multiple foci. The surface of the right lobe is focally disrupted (posterior/lateral). No lesions are identified in the right lobe or isthmus. Three possible lymph nodes (0.3 to 0.9 cm) are identified adjacent to the isthmus. The soft tissue inferior to the left lobe contains multiple possible lymph nodes (0.2 to 0.9 cm). Representative portions of the left lobe nodule and normal thyroid tissue are submitted to

SECTION CODE: B1-B5, representative sections of the left lobe nodule; B6, B7, representative sections of unremarkable left lobe tissue; B8, representative sections of isthmus; B9, three possible lymph nodes adjacent to isthmus;

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B10-B12, representative sections of right lobe; B13, five possible lymph nodes in the soft tissue inferior to the left lobe; B14, seven possible lymph nodes in soft tissue inferior to the left lobe.

(C) RIGHT NECK SEBACEOUS CYST - Received fresh is an unoriented ellipse of light tan skin (3.2 x 1.8 x 0.6 cm in greatest dimension) with two raised, light tan lesions measuring 0.5 and 0.2 cm respectively, resting 0.6 cm from each tip. The cut surface is marked with blue ink. Specimen is serially sectioned revealing a smooth wall cystic structure filled with yellow-tan cheesy material. The specimen is submitted with tips in cassette C1. The remainder of the specimen is submitted in cassettes C2 and C3.

CLINICAL HISTORY

Thyroid cancer.

SNOMED CODES

T-B6000, M-80503

"Some tests reported here may have been developed and performance characteristics determined by the U.S. Food and Drug Administration."

. These tests have not been specifically cleared or approved

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ADDENDUM

Addendum completed by:

DIAGNOSIS

(B) ISTHMUS:
METASTATIC CARCINOMA IN THREE LYMPH NODES (3/3) (SEE COMMENT).

COMMENT

Immunoperoxidase studies for pan keratin and cytokeratin 19 show scattered microscopic foci of metastatic carcinoma in three of three lymph nodes associated with the isthmus. No extracapsular extension is identified.

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